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06.10 Introduction

Service planning encompasses a comprehensive review of the participant's strengths, resources, needs, and conditions. The Case Manager will assist the participant in setting goals based on the participant's desires. The service planning process is determined by the participant and/or Authorized Representative in conjunction with the SCDHHS worker and/or Medicare – Medicaid Plan (PRIME) designee and allows for participation of the physician, service providers and other individuals identified by the participant and/or Authorized Representative. Service planning provides the participant with information necessary to make informed choices regarding the location of care and services to be utilized.

Service planning includes service coordination with other agencies such as home health, case management, or other involved entities, etc., to ensure all services (waiver and non-waiver) are considered in the development of the service plan. (Refer to Section 05.34)

Completion and implementation of the service plan is a function of the Case Manager and all involved parties.

06.20 Service Plan Policy and Procedure

06.21 Development of the Initial Service Plan

Service Plans must be person-centered. Prior to the development of the Service Plan, the participant and/or Authorized Representative must be contacted by the Case Manager to facilitate a discussion regarding the participant's strengths, preferences, goals and needs. Initial service planning begins during the waiver enrollment transfer conference held with the Nurse Consultant. The Service Plan is developed by the **enrolling Case Manager** in conjunction with the **Nurse Consultant**. The **enrolling Case Manager** and **Nurse Consultant** must discuss the assessment information and include information gathered from the team conference, participant, and/or Authorized Representative's input, and agencies providing services to the participant. Once the assessment information is discussed, it is the **enrolling Case Manager's** responsibility to document the discussion in a narrative. This information will assist the **enrolling Case Manager** in identifying the participant's preferences, needs and goals.

Several factors influence the service planning process and the determination of the services to be authorized. These factors include:

1. The participant's expressed need for assistance and the value assigned to that need.
2. The amount and type of assistance in place.
3. The degree of assistance, and the time required to complete the needed care.
4. The participant's cognitive ability, strengths, and preferences.
5. The availability of other cost-effective resources to meet these needs.
6. The participant's enrollment in Hospice. (Please refer to Hospice Benefit Interactions located in Phoenix HELP.)
7. The participant is residing in a Community Residential Care Facility. (Please refer to Waiver Interactions with Community Residential Care Facility located in Phoenix HELP.)

Active participation and planning with the participant and/or the Authorized Representative regarding the participant's long-term care is an integral part of the CLTC Program. Development of a realistic and thorough Service Plan and its implementation in the community involves numerous contacts and extensive planning and coordination.

Service planning must address the participant's desires, strengths and needs identified through the assessment process as well as viable solutions. It must include resources currently utilized by the participant, both waiver and other, as well as additional services which may be available to meet the participant's needs. All payment sources, where

appropriate, should be considered prior to using Medicaid services (including waiver services) in the Service Plan.

The Service Plan must be updated on an ongoing basis as the participant's goals, strengths, resources, preferences or needs change.

06.22 Purpose of the Service Plan

The results of the service planning process are documented on the Service Plan. It is the intent of the Service Plan to be a multidisciplinary plan rather than a nursing care plan. A completed Service Plan is required for all waiver participants.

Each Service Plan should be individualized for a participant and completed so that a service professional unfamiliar with the participant can have, by reading the plan, a clear picture of the participant's needs, strengths, preferences, planned interventions, and person(s) performing the interventions.

06.23 Elements of the Service Plan

The Service Plan includes seven (7) elements:

- Personal Goals
- Problems/Needs
- Goals
- Strengths
- Waiver/ Non-Waiver/Caregiver Supports
- Planned Services and Interventions
- Service Plan Participant Agreement

The service plan is expected to address all assessment components identified as an active need or goal. The service plan Comments Sections should be used only for insertion of clarifiers and should not be used as a Narrative.

The initial Service Plan must be completed within seven (7) business days of the participant's waiver enrollment.

EXCEPTION: Since the person-centered planning process requires the participant and/or Authorized Representative to select the time and location and whom they wish to be involved in the Service Plan development, the Case Manager may not always be able to meet the seven (7) business day timeliness standard. Any exceptions to this time frame and the

reasons for the delay in Service Plan development must be at the request of the participant and/or Authorized Representative, approved by the Lead Team Case Manager, Lead Team Nurse Consultant or Area Administrator and be documented in the narrative record.

06.23.10 Personal Goals

Personal Goals are determined by a discussion with the participant or Authorized Representative, if participant is unable. The **Case Manager** will ask if the participant has any Personal Goals and what they are. Personal Goals should be realistic and the steps to achievement may involve waiver supports or other means.

The **Case Manager** will list the steps identified by the participant and/or Authorized Representative to achieve this goal. The **Case Manager** must indicate when the steps are achieved or terminated.

At a minimum, the participant's Personal Goals must be addressed at the initial visit, quarterly visit, and re-evaluation visit. It may also be appropriate to address these during monthly contacts. The Personal Goals must be clearly documented in the Service Plan.

Personal Goals are an important part of person-centered planning and reflect what is important to the participant. All goals should be SMART:

- Specific
- Measurable
- Achievable
- Relevant
- Time-limited

Personal goals must be:

1. Stated in positive terms, not in terms of what should be avoided;
2. Defined in terms of the expected outcome (a result or condition to be achieved rather than an activity to be performed);
3. Written in quantifiable (measurable) terms, so that all involved persons may know when the goal is reached;
4. Achievable, taking into consideration known resources;
5. Designed as a joint commitment between the participant and the **Case**

Manager, based on the participant's wishes and priorities; and,

6. Written to achieve a single end, not a conglomerate of expected outcomes.

06.23.11 Needs

In order to develop a plan for intervention, a need must be clearly defined. The Service Plan must address all areas in which the participant requires at least limited/moderate assistance. Each assessed need with corresponding goals will be populated on the Service Plan. All identified needs/goals should be those which the **Case Manager**, participant, and Authorized Representative will be actively working to meet.

When the **Case Manager** identifies services that are needed but are unavailable, these services should be noted in the body of the Service Plan as a need and identified as an unmet need under the intervention. If an identified service need is not populated on the Service Plan, custom needs, goals, and intervention must be added.

Each listed need must have corresponding goals and interventions.

06.23.12 Strengths

Identifying a participant's strengths is an important part of the assessment/service planning process. When thinking about content to enter, consider strengths that help the participant remain in the best possible health while living in their chosen setting.

Examples of strength areas include but are not limited to:

- Social relationships/connections
- Family supports
- Mental health
- Self-care skills
- Talents/interests
- Education/training

06.23.13 Caregiver Supports and Non-Waiver Supports

The Caregiver Supports and Non-Waiver Supports sections of Phoenix contain information regarding the family and other non-waiver caregivers assisting the participant in the community. All corresponding fields in Phoenix should be completed.

The participant or Authorized Representative must give permission in order for the **Case Manager** to consult other involved caregivers during the service planning process. If this permission is initially obtained verbally, this must be documented in the narrative. The participant should sign the Appointing an Authorized Representative Form 1282 LTL at the next in-home visit, if capable. If not capable, the Authorized Representative should sign. If neither the Authorized Representative nor the participant is capable, the **Case Manager** may sign the form. If the **Case Manager signs**, an explanation must be written in the comment section of Caregiver Supports section.

The following should be completed in the Caregiver Support Section of Phoenix:

1. Name of the caregiver;
2. Relationship to the participant;
3. Tasks completed by the caregiver and an indication of whether or not the caregiver is the primary caregiver for the task;
4. Frequency - how much of the care the support person is able to complete for the participant;
5. Address and Phone number(s) of the caregiver;
6. Dates of task coverage;
7. Interview of caregiver support indicated as primary for the task.

NOTE: **Caregiver Interviews are to measure the primary caregiver's stress level.** When a caregiver is performing the "Hands on Care" task and is the "Primary" caregiver, an interview questionnaire (Caregiver Interview) **MUST** be completed at the initial visit then during each subsequent re-evaluation. However, this questionnaire can be updated any time during the service plan year if the caregiver exhibits an increase in stress.

A Caregiver Supports reference guide is available under Phoenix Help Documents.

06.23.14 Waiver Supports

The Waiver Support Section in Phoenix is used to house information regarding waiver services in place for the participant. It is also used to generate Service Provision Forms (Authorizations) for waiver services.

06.23.15 Planned Interventions

Once a need/goal has been established, interventions should be selected to reach the goal.

In general, interventions are actions taken to improve a situation. The appropriate interventions must be chosen to address each need/goal within each section of the Service Plan.

06.23.16 Service Plan Participant Agreement

The Service Plan Agreement Section is part of the current Service Plan and is presented to the participant and/or Authorized Representative as the last page of the service plan. The Service Plan Agreement Section serves as a record that the Service Plan has been developed, reviewed, and evaluated with the participant and/or Authorized Representative. The Service Plan Agreement Section is signed and dated at **the Initial Visit AND during the participant's first quarterly visit following a re-evaluation** visit. The dated Service Plan Agreement Section is signed by both the **Case Manager** and the participant and/or Authorized Representative. It is used on an on-going basis and is maintained in Phoenix.

If a hard copy Service Plan Agreement Form is needed, the signed hard copy form must be scanned into Phoenix Scans under the “Service Plan Agreement” scan tag. A copy of the Service Plan must be given to the participant and/or Authorized Representative at the first in-home visit following the creation of a new Service Plan. If revisions to the Service Plan are needed following the in-home visit, a copy of the revised Service Plan must be mailed to the participant and/or Authorized Representative, as long as an Authorized Representative Form 1282 CLTC is on file to share information with someone other than the participant.

06.24 Service Plan Components

These are different “TABS” in the Service Plan that correspond to the tabs on the assessment or to a different module in Phoenix. There are seven (7) components identified on the Service Plan. These components have applicable information which can be identified through the assessment instrument as follows:

- Medical
- Skin/Nutrition
- Activities of Daily Living (ADLs)
- Instrumental Activities of Daily Living (IADLs)
- Psychosocial

- Caregivers
- Home Assessment

06.24.10 Medical

Diagnoses are auto populated on the Service Plan from the assessment information entered into Phoenix.

06.24.11 Psychosocial

If the waiver participant indicates that he or she is experiencing social isolation or withdrawal from leisure activities as new or existing behaviors, the Case Manager should discuss these behaviors with the waiver participant and document responses in Phoenix.

A Custom Needs/Goals/Interventions must be entered on the Service Plan for socially isolated or withdrawal from leisure activities behaviors.

06.25 Initial Service Plan Completion and Implementation

The initial Service Plan must be completed within seven (7) business days of waiver enrollment. The Service Plan is complete upon team staffing and signing.

Once completed, the Service Plan is available to all appropriate waiver service providers in Phoenix, except Attendants. The Case Manager will provide a copy of the Service Plan to Attendants after the completion of the Attendant Care Training. If training is not required, the Case Manager will mail a copy of the Service Plan to the Attendant.

06.26 Service Plan Revision within 365 Calendar Day Cycle

The Service Plan may require revisions when changes in the participant's condition, service needs and/or preferences occur which do not affect the re-evaluation cycle itself. Any additions or revisions in the Service Plan, including waiver, caregiver supports, and other supports, within the re-evaluation cycle must be entered into the Service Plan. Upon receipt of a request to modify the service plan, the Case Manager will discuss the request with the participant and/or Authorized Representative and review/modify the service plan within two (2) business days.

When additions or revisions are made to the Service Plan during the 365-calendar day cycle, the affected provider(s) must be notified of the Service Plan changes via phone call or Conversation feature in Phoenix. Attendants must be notified by phone or CLTC Notification Form.

For service needs/goals, interventions, or actions that are no longer appropriate, comments should be added to the identified need/goal to indicate the change and the effective date of the change, if appropriate. For example, if an intervention is no longer being used, a comment should be entered noting such.

06.27 Team Involvement

Service planning is always a team effort. Staffing for levels of care and service plan reviews at enrollment and re-evaluation must be completed with the SCDHHS worker (or the Medicare – Medicaid Plan (PRIME) designee for PRIME waiver participants) in a manner where both parties can see the most current information in Phoenix. (This may be the **Case Manager II, Lead Team Case Manager, Area Administrator, Lead Team Nurse Consultant, Nurse Consultant or Medicare – Medicaid Plan (PRIME) designee.**) Team signatures are completed in Phoenix.

06.28 Service Plan at Re-evaluation

Ensuring the Service Plan's effectiveness and accuracy is an on-going process.

At re-evaluation, the new Service Plan Interventions should reflect only the current services received. Service Plans must be person-centered. The participant and/or Authorized Representative and any other parties which the participant and/or Authorized Representative choose to be included in the service planning process must be contacted for their input in evaluating the effectiveness of the Service Plan, and any changes that have occurred in the participant's condition or support system. It may be appropriate to include the physician, providers, home health nurse or others involved in the participant's care.

Service Plan re-development must be completed within three hundred sixty-five (365) calendar days of the last assessment date. Service Plans are to be re-developed at the same time as the assessment and have the same effective date. **If the assessment is greater than thirty (30) days from Service Plan completion date, a new Assessment must be completed.** The Service Plan must be created, completed, staffed, and signed in Phoenix, and all data must be uploaded to Phoenix, by the three hundred and sixty-fifth (365th) calendar day of the previous assessment date. The completed assessment for re-evaluation visit, Level of Care, and Service Plans must be completed by the due date.

Example:

Current Level of Care and Service Plan Date	Next Level of Care and Service Plan Completion Due Date
August 15, 2020	August 14, 2021

A participant who experiences a significant change in condition requires an immediate review of the Service Plan. Participants who experience frequent changes impacting service planning, may require Service Plan review more frequently than every 365 calendar days. A participant with a significant change requiring a new assessment, also requires a new

Service Plan.

The appropriate providers, except Attendants, receive a Phoenix Notification when a new Service Plan has been completed following a re-evaluation. The Case Manager should mail the Attendant a copy of the new Service Plan when completed.